Hospital Formulary 2026-2027

Reference	PHA-FOR-00 v2026
Owner	Drug and Therapeutics Committee
Approved by	Quality and Patient Safety Committee
Effective date	01/07/2026
Next review	01/07/2029
Applies to	All clinical areas of the Centre Hospitalier Saint-Aubin

1. Status codes

F - formulary: may be prescribed by any authorised prescriber. **R** - restricted: initiation only by the named specialty; continuation by any prescriber. **NF** - non-formulary: supplied only after individual patient approval by the Drug and Therapeutics Committee (D&T); the approval states its validity period.

2. Formulary list

Medicine	Status	Condition
Allopurinol	F	
Amlodipine	F	
Amoxicillin	F	
Apixaban	F	
Aspirin	F	
Atorvastatin	F	
Bisoprolol	F	
Cefazolin	F	
Ceftriaxone	F	
Clarithromycin	F	
Dapagliflozin	R	cardiology or diabetology consultant initiation
Denosumab	NF	individual patient approval by D&T
Digoxin	F	
Doxycycline	F	
Enoxaparin	F	
Eplerenone	R	cardiology consultant initiation
Furosemide	F	
Gentamicin	F	
Ibuprofen	F	
Insulin aspart	F	
Lansoprazole	NF	pantoprazole is the formulary PPI alternative
Levothyroxine	F	
Linaclotide	R	gastroenterology consultant initiation
Metformin	F	
Omeprazole	F	
Ondansetron	F	
Pantoprazole	F	
Paracetamol	F	
Pregabalin	F	
Pregabalin XR	NF	use standard-release pregabalin
Ramipril	F	
Rivaroxaban	NF	individual patient approval by D&T
Sacubitril/valsartan	R	cardiology consultant initiation
Sertraline	F	

Medicine	Status	Condition
Simvastatin	F	
Spironolactone	F	
Ticagrelor	R	cardiology consultant initiation
Tolvaptan	NF	individual patient approval by the Drug and Therapeutics Committee (D&T)
Tramadol	F	
Warfarin	F	

3. Governance, audit and training

This document is controlled. Printed copies are uncontrolled: the current version is held on the hospital document management system and a printed copy must be checked against it before use.

The document owner reviews the content at least every three years or earlier when national guidance, a serious incident or an audit finding requires it. Interim changes are approved by the relevant committee chair and recorded in the revision history.

Compliance is audited by the clinical audit department on a rolling sample of records. Results are reported to the quality and patient safety committee, which agrees actions with the ward managers and monitors their completion.

Staff who identify a deviation from this document, or a situation it does not cover, should discuss it with the senior clinician on duty and report it through the incident reporting system. Reporting is a learning tool and not a disciplinary process.

4. Revision history

Version	Date	Author	Summary of change
1.0	01/07/2021	Drug and Therapeutics Committee	First issue
2.0	01/07/2023	Drug and Therapeutics Committee	Triennial review; format harmonised
3.0	01/06/2025	Drug and Therapeutics Committee	Tables updated after clinical audit
current	01/07/2026	Drug and Therapeutics Committee	Review and re-issue

Annex A. Implementation notes

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Equality and diversity: this document has been assessed for its effect on patients and staff with protected characteristics. Reasonable adjustments, interpreters and accessible formats are available through the patient experience team.

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Data protection: any patient information handled under this document is processed in accordance with the data protection policy of the hospital. Access to records is limited to staff involved in the care of the patient.

Training on this document is part of the induction programme for clinical staff and is refreshed through the annual mandatory training cycle. Agency and locum staff receive a short briefing at the start of their first shift.

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Annex B. Implementation notes

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